

Plot/Intro [00:00:03–00:00:36]

Welcome to The Lighter Side of the Spectrum, the podcast that celebrates the multiple hats we wear and the juggling acts we perform in life. Whether you're parenting kids and teens on the autism spectrum, navigating the ever-evolving healthcare industry, or building a more inclusive and impactful world, this is your space to find connection, inspiration, and growth. Together, we'll share stories, tackle challenges, and explore how innovation and inclusion create better outcomes for everyone. Because no matter the spectrum, there's always a lighter side. Now here's your host, David Kaufer.

David Kaufer [00:00:36–00:00:49]

Hello again, everybody. This is David Kaufer. Welcome back to The Lighter Side of the Spectrum, where we explore the people, ideas, and stone.

Denise Allen [00:00:49–00:00:49]

Hi.

David Kaufer [00:00:49–00:02:33]

Hi. How are you? Good, good. Happy, happy. Okay, okay.

And conversations that help us better understand autism communication and what it really means to support human potential. You know, one of the things I've certainly learned over the last few years is that so many of the assumptions that we make about people turn out to be wrong. We've learned that what looks like a lack of understanding may actually be a motor planning challenge. We've also learned that what looks like a behavioral issue may be sensory overload. What appears to be an inability to communicate may simply be a person who hasn't been given the right way to communicate. Today's conversation is going to ask us to consider another assumption. What if vision isn't just about seeing clearly? You know, most of us think of vision as reading, reading the eye chart at your doctor's office, trying to see if you have 20/20 vision. You know, if you're good, you're good to go. But what if your eyes are healthy, yet your brain isn't using them together in the way it's meant to be?

What if the issue isn't how well you see, but how you see? And that's the fascinating world we're going to step into today. And my guest today is Denise Allen, host of Healing Our Sight podcast and a passionate advocate for helping people better understand binocular vision and the remarkable adaptability of the human brain. Denise spent most of her life with strabomide— strabomys— I knew I was gonna screw it up. Well, help me, Denise.

Denise Allen [00:02:33–00:02:34]

Strabismus.

David Kaufer [00:02:34–00:03:35]

Strabismus. I even practiced it. And without stereoscopic or 3D vision, like many people, She assumed that was simply the way her brain worked. But as an adult, she discovered that our visual system may be far more adaptable than many of us realize. And that journey led her into the worlds of vision therapy, neuroplasticity, developmental optometry, and neuroscience.

Along the way, she's had conversations with leading experts while helping parents, patients, and professionals better understand conditions. And related to vision-related challenges. So this isn't about autism specifically, but we certainly see it within autism. And I think it's really going to be a fascinating conversation because vision, sensory processing, motor planning, communication, they're all parts of an incredibly complex system. So Denise, with all that being said, thanks for joining me today. I'm really looking forward to this conversation.

Denise Allen [00:03:35–00:03:37]

Thanks for having me.

David Kaufer [00:03:37–00:03:46]

Yeah. So why don't you tell us a little bit about your experience? What, what, tell us about what is it like to grow up with this condition?

Denise Allen [00:03:46–00:03:53]

Well, you know, I didn't know anything different, so I thought I was pretty normal.

David Kaufer [00:03:54–00:03:54]

Okay.

Denise Allen [00:03:55–00:04:17]

But I did have struggles with coordination, being very clumsy, not very good at catching Or, you know, balls or anything, any moving object coming at me was difficult to gauge where it was and catch it. So I was really bad at ballgames. So I was usually chosen last, you know, that kind of thing.

David Kaufer [00:04:17–00:04:18]

Right.

Denise Allen [00:04:18–00:04:28]

That posed a problem, of course. My knees were always scraped up, you know, I fell down. I just thought I was a clumsy kid, you know.

David Kaufer [00:04:28–00:04:34]

Yeah. So you just thought you were clumsy. I mean, right? Like, You hear about that?

Denise Allen [00:04:34–00:05:13]

So my mom put me in dance class. That was supposed to help. Didn't, didn't love it that much. No, no, it did help. And then I actually did gymnastics in junior high, which I shouldn't have probably been able to do very well. I was too tall for it. Wow. But yeah, that's challenging. It doesn't stop someone from doing something that they want to do. My brother, who had the same condition, only his showed up when he was a baby, he became a basketball player. He played college basketball, which also shouldn't really be very easy. But he was passionate about basketball.

David Kaufer [00:05:13–00:05:14]

Wow.

Denise Allen [00:05:14–00:05:23]

So he overcame that. So I think we don't have to discount anything, right? But the thing that I found is that it can be—

David Kaufer [00:05:25–00:05:46]

That's just— yep, Stone. Stone's very excited today. So again, he's making, making lots of appearances. He's getting, getting ready to head out with his brother. But that's amazing though. I mean, that he was able to play basketball, because again, why you— I mean, we'll back up a little bit and maybe kind of talk about, um, strabismus. Is that— did I get it better?

Denise Allen [00:05:46–00:05:47]

Yeah.

David Kaufer [00:05:47–00:05:56]

Okay. Um, What is it, you know, and how is it defined and how does it impact people?

Denise Allen [00:05:56–00:06:19]

Yeah, so strabismus is when the eyes don't focus together. And the easy explanation is just that they're fighting with each other all the time. And in order to compensate for that, one of the eyes will shut off. And when you look at someone, Often that eye is pointing in another direction.

David Kaufer [00:06:20–00:06:20]

Okay.

Denise Allen [00:06:20–00:06:37]

Because it's getting out of the way of the other eye that's the strong eye. So you'll see kids with an eye turn, right? Crossed eyes, however you want to term it, right? It can be turning in or it can be turning out. People will call it a lazy eye.

David Kaufer [00:06:37–00:06:41]

Yeah, you hear about that. Okay.

Denise Allen [00:06:41–00:07:03]

So that's kind of And so it's like 5% of the population that have an actual strabismus like this, of the normal population. If you're looking at the autistic spectrum, then it's totally different. And that's just

strabismus. That's not other vision problems like inability to track or—

David Kaufer [00:07:03–00:07:05]

Right.

Denise Allen [00:07:05–00:07:13]

Other focusing types of issues like that. You know, so my Mine is kind of a subset, but it's a much bigger problem than that.

David Kaufer [00:07:13–00:07:42]

Is this something that we're just, like so many other things, we're just learning more about? I mean, is there just— is it one of those we're just getting more awareness? Because growing up, I mean, what was— like you said, you were just kind of considered maybe a little clumsy, or you went in for traditional eye exams, right? Like, you know, was it just— what did the eye doctors see or say, you know? And what did it take for you to finally Yeah, that is a great question.

Denise Allen [00:07:42–00:08:02]

And I, and I think that it's always been a problem. I just feel like the fact that we can do something about it hasn't been very obvious. So when I— my brother, it was obvious at birth. He went into the doctor, they did surgery on his eyes because it was pointing way in, you know.

David Kaufer [00:08:02–00:08:06]

So it was so obvious that like, they're like, okay, we got to fix this. I mean—

Denise Allen [00:08:06–00:08:20]

Exactly. Yeah. And the traditional solution When you have this kind of an eye turn is the ophthalmologist will say, well, we're going to do surgery. They'll do 2 or 3 surgeries. Often it takes more than one. My brother had 3 surgeries.

David Kaufer [00:08:20–00:08:20]

Oh, wow.

Denise Allen [00:08:20–00:09:07]

Yeah. 2 when he was a baby and one when he was like 7 to kind of tune it up. So it's kind of a lot of surgery for a young child. It doesn't teach the eyes how to work together though, to have surgery. So even though my brother's eyes were fairly straight, He didn't learn how to see depth.

My problem didn't surface until I was almost 3. So it's a different type of strabismus. There's infantile, like my brother, and then there's a compensatory type of strabismus. And it usually means that you have either farsighted or nearsightedness, and your eyes are not working together and they're compensating. And they put glasses on the child like they did me, and the eyes look pretty straight.

And the doctor says, okay, you're good to go.

David Kaufer [00:09:09–00:09:11]

Just because your eyes were straight?

Denise Allen [00:09:11–00:09:45]

Exactly. And I remember as a child going in for my yearly exam, and I was about 10. And they did this test where you look in the big machine. And on one side, there's a note, and on the other side, there's the 5 lines for the music staff. And they said, can you tell us what line or space that note's on. And I said, well, I see the notes over here and I see the lines over here. Do you want me to guess? And I had been guessing until then.

David Kaufer [00:09:45–00:09:46]

Oh, really?

Denise Allen [00:09:46–00:09:58]

Something just clicked. And I thought— and I was annoyed because I'd had a traumatic event on the way to the office. And so, I think I was probably a little more sassy than usual, you know?

David Kaufer [00:09:58–00:10:00]

Yeah, a little edgy, a little edgy 10-year-old.

Denise Allen [00:10:02–00:10:15]

And I just said, do you want me to guess? Because I can't—I, I see them on, you know, with each eye. And, and I had never been able to see through binoculars or, you know, the little kaleidoscope thing that—

David Kaufer [00:10:15–00:10:17]

Oh yeah, yeah, those were fun.

Denise Allen [00:10:17–00:10:23]

Yeah, yeah, I could never do those. Um, and, and they just said, oh, you can't do that, we won't do that anymore.

David Kaufer [00:10:23–00:10:26]

That was their answer?

Plot/Intro [00:10:26–00:10:26]

Yeah.

David Kaufer [00:10:27–00:10:29]

Oh, great. That's helpful.

Denise Allen [00:10:29–00:10:44]

Yeah. And, you know, vision therapy has been around that long, but nobody knew about it. I didn't know about it until I went looking for answers as a, you know, 40-something-year-old because my eyes were bothering me more at that point.

David Kaufer [00:10:44–00:10:50]

Well, I think there's still a real lack of understanding about vision therapy.

Denise Allen [00:10:50–00:10:52]

Absolutely.

David Kaufer [00:10:53–00:11:26]

Yeah, it was something I had heard about again in our journey with autism ourselves. It's just one of many avenues that you explore when you're dealing with communication. And I remember hearing about—we have a place, I'm in the Seattle area, and checking out, but that was all new, just the whole idea of doing vision therapy. Because again, it's so—you just think about it, you just get eye exams and you get glasses. You don't think about this as something, another area, although it makes perfect sense once you start looking at it a little more closely.

Denise Allen [00:11:26–00:12:11]

Right. Yeah. And I didn't start looking until my eye turn started to become really bad in my 40s. I started to get presbyopia like a lot of us do at that age, you know, can't see till you're, you know, have your hand clear out here. And so my eye turn just got worse.

And, and I looked at natural vision improvement things. And I I started searching online and I just by accident, not by accident, right? Came across vision therapy, found out there was a doctor just down the street from me. And there were only 2 in the whole state of Utah at that point. And went in and had an exam and figured out that there was something we could do.

David Kaufer [00:12:11–00:12:29]

Wow. Well, that's, I'm glad that you found that. What was it like then? I mean, what and how? Tell me about that experience. As you—after you went through, started going through the therapy, did you experience like true depth perception for the first time, or, you know, things like that?

Denise Allen [00:12:29–00:12:31]

Well, it wasn't quite that easy.

David Kaufer [00:12:31–00:12:32]

It wasn't like—

Denise Allen [00:12:32–00:13:08]

Yeah, no, fair. Um, it, it was, uh, about 2013. I started vision therapy. Did a lot of sessions of therapy, and I just wasn't getting any resolution at all. I wasn't getting any progress. And my doctor said, you know, you can look at surgery. That could be what you might want to do in addition. And I didn't want

to do surgery, so I stopped vision therapy, took my daughter in, found out that she was mini-me and that she needed vision therapy. Wow.

David Kaufer [00:13:09–00:13:09]

Okay.

Denise Allen [00:13:10–00:13:11]

Well, and I knew—

David Kaufer [00:13:11–00:13:13]

It's really genetic, it sounds like.

Denise Allen [00:13:13–00:13:53]

Yeah, it's hard to say. Yeah, it's hard to say. But I mean, I have 6 children. My youngest daughter was just like me. I knew she was like me because she had had an eye turn when she was 3, like I did. And I had taken her to the doctor and they put her in glasses. It was just, you know, the same process. And she was fine. She did fine in school. She didn't seem to struggle that much in other things. She didn't seem to be super clumsy. I had done other things with her when she was very young, like music and movement. I taught music and movement classes. And so there were things that maybe compensated better. I don't know.

David Kaufer [00:13:53–00:13:53]

Right.

Denise Allen [00:13:53–00:14:29]

And I didn't see her doing poorly in school or having any struggles. And I had this passion for sharing the whole thing, you know, because I knew it could help. It hadn't helped me yet, but I knew it could. And so I was reading this book called When Your Sorry, When Your Child Struggles by David Cook. And it had a checklist in it of things to ask the child. And I was thinking about it in relationship to children that I was experiencing at school when I was substitute teaching. But I practiced.

David Kaufer [00:14:29–00:14:29]

Oh, okay.

Denise Allen [00:14:29–00:14:50]

I decided to practice the questions on my daughter. And so I said to her, do the words ever do this when you're reading? And, you know, they split apart. And she said yes. And I thought, oh, that's not good. I had no idea that she was seeing double.

David Kaufer [00:14:50–00:14:52]

And how old was she at this point? Remind me.

Denise Allen [00:14:52–00:14:54]

She was 9.

David Kaufer [00:14:54–00:14:55]

Okay. So about— okay.

Denise Allen [00:14:55–00:15:37]

Yeah. Yeah. So very starting to be very aware of that and potentially have problems later on, you know. Because reading just— the words just keep getting smaller, right? And more difficult. So I said, well, do you ever close an eye when you're reading? That was also on the checklist. And she said, oh yeah, all the time. When I'm reading in bed, I lay with my eye on the pillow so that I'm reading just with the one eye. And she said, and then I turn over when I turn the page and do it with the other eye. So that was helping avoid double vision, right? And I had—

David Kaufer [00:15:37–00:15:45]

Yeah, like she kind of figured it out. Like she had her own solution. I mean, it was a coping mechanism, basically.

Denise Allen [00:15:45–00:16:43]

Exactly. Yes. And I don't know if she did any of that at school. Sometimes children will, you know, they'll sit with their hand up like this, you know, cover an eye and just make other compensatory items

that will help them to not have that problem. So she was doing all of those things.

I had no idea. And I thought I was a pretty aware mom. She was trying to have kids. I'm sure you were. Yeah.

Right. Right. I was busy, but I kind of was beating myself up a little bit about it. So I took her into my vision therapy doctor, and he got her in vision therapy. It took about 9 months of therapy for her, and she was able to gain 3D. So it's a pretty easy process.

It is something for a lot of people that you kind of have to keep doing a little bit of to keep the skill.

David Kaufer [00:16:43–00:16:45]

Kind of like exercise in a way.

Denise Allen [00:16:45–00:17:02]

A little bit, yeah. So I think she might have lost a little ground. We've talked about that over the years. She's been on my podcast too. And told her story. So she recognizes that there might be some things she could do to kind of strengthen her ability still. And in the meantime, my doctor—

David Kaufer [00:17:02–00:17:05]

It takes time and effort to do that. It does.

Denise Allen [00:17:05–00:17:25]

Yeah, it does. And for some people, it takes way longer, you know, and there are cases that take less time. But I think 9 months is a fairly standard or comfortable amount of time that it will take for people, if they're— for kids, you know, kids are much more malleable than adults are. So—

David Kaufer [00:17:25–00:17:26]

Oh, yeah, absolutely.

Denise Allen [00:17:26–00:17:58]

I went back in at the end of that process. My doctor had done some training in some other kinds of therapy. I went in, I tried those, and still wasn't getting the results I wanted. At this point, I'm like 80 sessions into vision therapy. And once again, he said, you probably need to look at surgery. So he referred me to a doctor that he's worked with. I went in, I had the strabismus surgery.

David Kaufer [00:17:59–00:17:59]

Okay.

Denise Allen [00:17:59–00:18:08]

You know, don't know what that looks like. It's, you know, you go in, they cut all the muscles and they move them so that your eyes are lined up instead of crossed.

David Kaufer [00:18:08–00:18:29]

Wow. I mean, that's a big, big step. I could see why you, you know, you would want to avoid it if at all possible. I think anytime For most of us, right? You talk about our eyes or eye surgery, it's just, it's different. It hits different than just saying, oh, you know, you, we got to do some elbow surgery, for example, or something like that.

Denise Allen [00:18:29–00:18:35]

Well, I mean, when you're cutting a muscle, I think you tend to hesitate before you're—

David Kaufer [00:18:35–00:18:46]

It's your eyes. It's your eyes. All right. Like, that's very, very personal to all of us. So, but it sounds like it was It sounds like obviously it was a success.

Denise Allen [00:18:46–00:19:09]

Yeah, it was amazing. I had a really easy time with the surgery. I took like one pain med. I decided the pain meds weren't really helping me, not doing good things to my body. I said, not doing that. And so I'm really into natural remedies. So I took this homeopathic remedy called Arnica.

David Kaufer [00:19:10–00:19:11]

Oh, okay.

Denise Allen [00:19:11–00:19:22]

Instead of any of the other drugs they had given me. And I went back for my checkup a week later. And

my eye doctor said, I can't believe how fast you've recovered from this surgery.

David Kaufer [00:19:22–00:19:23]

Wow.

Denise Allen [00:19:23–00:19:48]

I said, great, thank you. And that week, I also went back into vision therapy. And they don't necessarily recommend that you do that. You know, a lot of times doctors will say, oh, you should, Recover first or whatever. I mean, I was recovered, but not everybody will go back in that quickly. And as soon as I went back in, I could do everything that I hadn't been able to do before.

David Kaufer [00:19:48–00:19:52]

Wow. Wow. That must have been amazing.

Denise Allen [00:19:52–00:20:05]

It was. Yeah. And I gained, you know, pretty good depth perception, especially close up. My depth perception is in normal range. Further out, it's not quite as good. So I'm still working on that.

David Kaufer [00:20:05–00:20:10]

And I'm just curious, do you have 20/20 vision as well now, or do you—

Denise Allen [00:20:10–00:20:17]

There's no relationship between the way that the eyes focus together and the—

David Kaufer [00:20:17–00:20:18]

The vision.

Denise Allen [00:20:18–00:20:30]

The acuity piece. The acuity piece. All right. We think that vision is just all about seeing 20/20, and it's not at all. So vision therapy doesn't fix that piece.

David Kaufer [00:20:31–00:20:31]

Okay.

Denise Allen [00:20:31–00:20:33]

I still need glasses.

David Kaufer [00:20:33–00:20:33]

Okay.

Denise Allen [00:20:33–00:20:54]

So, I mean, I wear contacts, but yeah, that piece doesn't get fixed by therapy like that. And there are other things that we can do to improve the acuity piece, but that's not typically done in a regular doctor's office. You know, that kind of goes into the realm of natural vision improvement.

David Kaufer [00:20:54–00:21:07]

Okay. But that's remarkable that you recovered that quickly. Do you attribute it to the the homeopathic approach that you took, or just your, you know, were there other steps that you took that maybe helped accelerate it?

Denise Allen [00:21:07–00:21:33]

Uh, I think that that's what I've typically attributed it to, but I also am really healthy, and mindset also plays into it. The fact that I had never had a surgery before made it easier. You know, people who've had other strabismus surgeries, there's scar tissue that it's going to get in the way and it makes it a harder process.

David Kaufer [00:21:33–00:21:52]

No, that makes a lot of sense. You know, earlier you were talking about how it affects, you know, roughly 5% of the general population, but that the percentage is a lot higher when you're talking about the neurodiverse or the autistic population. Do you have any estimates of how high it is?

Denise Allen [00:21:52–00:21:58]

You know what, I thought you might ask me that. So I pulled out this book that I like. It's called— can you see it?

David Kaufer [00:21:59–00:22:00]

Yeah, I guess, uh, yeah, kind of.

Denise Allen [00:22:00–00:22:28]

The Hidden Link Between Vision and Learning by Wendy Rosen. She's one of my podcast guests, and, and I— oh great, I knew she had told me this, and, or that I had read it, and so I looked it up. And, um, it says according to her that about 25% of children in the, in the United States struggle with learning problems, and then 25% of children also have a vision problem that impedes learning. So let's see maybe there's a connection there, right?

David Kaufer [00:22:28–00:22:29]

Mm-hmm.

Denise Allen [00:22:29–00:22:57]

And according to the National Center for Health Statistics, 20 to 25% of children enter school with significant vision problems. So, and that's, I mean, that's not just strabismus, that's other vision issues, right? The greatest difficulty among learning disabled children is with reading, which is affecting 75% of that population. And 80% of these children struggle with one or more visual skills.

David Kaufer [00:22:57–00:22:58]

Wow.

Denise Allen [00:22:58–00:23:02]

So that's a huge number.

David Kaufer [00:23:02–00:23:55]

That is a huge number. And as you're talking about it, I'm thinking, you know, as it relates to the autistic and the non-speaking community, is that how frustrating that must be. So, you know, because there are a lot of non-speakers who have gained communication through the letterboard or typing or, you know, other methodologies, that, that's when it finally comes out that they have vision issues. It could be double vision or, you know, similar types of issues. Of course, they had no way to share that before, right?

So they're, like you said, they're trying to, they're trying to cope. They're trying to cope with vision issues without having the way To share that, to communicate, like, hey, I'm— I can't see in 3D, or I can't— yeah, I'm seeing double of everything, you know, these words, or all of that.

Denise Allen [00:23:55–00:24:05]

Yeah, well, and I mean, everyone thinks that this is just the way you see. They have no idea that someone else sees differently than they do.

David Kaufer [00:24:05–00:24:11]

Right, right. Even when you're growing up, right? Growing up, you're like, oh, it's just the way— this is just the way it is.

Denise Allen [00:24:13–00:24:18]

Yeah, my daughter had no idea that anyone saw any differently than she did either, you know.

David Kaufer [00:24:18–00:24:33]

And she didn't think it was— I want to say weird, you know, but different. She didn't think anything about reading the way that she read, for example. Like, that was just the way it was. You just, you know, cover up one eye and roll over and do the other eye. And that's just the— yeah, that's just what you do.

Denise Allen [00:24:33–00:24:35]

Exactly. Yep.

David Kaufer [00:24:35–00:24:52]

So what role does neuroplasticity play in vision? I mean, what, you know, when it comes to this, like you said, it's not just, you know, looking 20/20. Are there— are the therapies— vision therapy, does that deal with issues with the brain in terms of training the brain as well?

Denise Allen [00:24:52–00:24:56]

That's exactly what it's doing. Yeah.

David Kaufer [00:24:56–00:24:56]

Okay.

Denise Allen [00:24:56–00:25:30]

We're, we're training the brain to see correctly because the eyes themselves are just an organ, right? They— the muscles are not going to do anything you don't tell them to do. So if the brain isn't telling the eyes what to do, you're not going to see properly regardless. You know, it really— vision really is just in the brain. And we don't think of it that way. We think our eyes are seeing, you know, but I mean, our arm's not going to move if our brain doesn't tell it to. So why do we think our eyes know what to do if our brain doesn't tell them what to do?

David Kaufer [00:25:30–00:26:03]

Why do we think that, Denise? That's a good question. I mean, because it's true. That's absolutely the perception. You don't think of it as a muscle. You don't think of it as something that your brain has to control, I guess, unless you're really in a situation when you— that kind of focus is required. Um, anything from like looking through a microscope or binoculars or, you know, whatever, where you're like, oh, you know, you're actually focused on the task at hand. I guess you just take it for granted. I mean—

Denise Allen [00:26:03–00:26:08]

We all take it for granted all the time. And do you—

David Kaufer [00:26:08–00:26:29]

Yeah, yeah, go ahead. Kind of like walking. Again, you, you take advantage— you, you take it for granted if you're able to walk, right? If you've gone through that progression, you know, that's all you know. You, you learn to crawl, you learn to walk. You want to go from point A to point B, you walk there. You don't think about your brain's telling your legs to move.

Denise Allen [00:26:29–00:26:31]

Exactly.

David Kaufer [00:26:32–00:26:34]

I guess it's the same for vision.

Denise Allen [00:26:34–00:26:56]

And if our eye muscles aren't working properly, it's no different than any other muscle in our body, you know, and we do exercise to strengthen those muscles or to relax those muscles, right? And in the case of our eyes, we really need to relax the muscles in order for them to see properly. And we never think of it that way either, you know.

David Kaufer [00:26:57–00:27:22]

No, not at all. I mean, you're starting to see some awareness in terms of eyes. I'm thinking about like with eye fatigue now with all the screen time, you know, things kind of related to that. Hey, you need to give your eyes a break. You know, you don't want to be staring at a screen for hours on end and, and all that. But I still feel like it's fairly shallow in terms of the approach that we take.

Denise Allen [00:27:22–00:27:25]

I agree. Absolutely.

David Kaufer [00:27:27–00:27:43]

Yep. Um, yeah, I was thinking also with, especially with kids and screen time, and, you know, this all overlaps, but could some, you know, could these problems sometimes look like ADHD or learning disabilities or sensory issues?

Denise Allen [00:27:43–00:28:10]

Absolutely. Yeah. And the fact that we see it in this population should point that out a little bit. You know, if the same kids that are struggling are the ones who have a vision problem, then, you know, which came first? And what's the best way to deal with it, right? And, you know, I have another favorite book that I wanted to share.

David Kaufer [00:28:10–00:28:11]

Yeah, please.

Denise Allen [00:28:11–00:28:38]

There's, well, there's a couple that were written by Robin and Jillian Benoit. One of them's called Jillian's Story. And then I have this one, Dear Jillian. And they talk in this book about— well, in the first one, she tells Jillian's story. And Jillian had amblyopia. She had passed every eye exam. And the way she passed it— and I don't know if you know what amblyopia is.

David Kaufer [00:28:38–00:28:39]

I do not.

Denise Allen [00:28:39–00:28:51]

Okay, well, it's a type of strabismus. Amblyopia, you have lower acuity in one eye. So if you can't see very well out of one eye, you're just going to use the other eye.

David Kaufer [00:28:51–00:28:51]

Okay.

Denise Allen [00:28:51–00:29:33]

And that eye just kind of shuts off. And sometimes if that happens for a long period of time, the person really goes blind in that eye. And it just kind of becomes— they treat it as a permanent kind of problem. I don't agree that any of that's permanent, but There's different perceptions on that. Anyway, Jillian had, um, pretty significant amblyopia, but she'd passed every test when they've done the eye exam at the school because she thought it was like a memorization test.

And so she would listen to the kids in front of her, and she wanted to get the treat, right? And so she would just rattle off whatever they said, even though she couldn't see it at all.

David Kaufer [00:29:33–00:29:40]

Wow. That's— I mean, it's ingenious, but Not helpful in terms of—

Denise Allen [00:29:40–00:30:03]

Exactly. Yeah. And so when she finally got into vision therapy, then, you know, she was able to overcome that and gain 3D vision and then tell her story, you know. And then all these people started writing to her saying, hey, let me tell you my story. And that's what went into the second book, right? And she did talk to someone that was autistic.

David Kaufer [00:30:04–00:30:05]

Oh, interesting.

Denise Allen [00:30:05–00:30:40]

Their family, right? And that's why I wanted to share this, because I think this is a really great story about it. Um, he— this, this child was only 2 when they figured out that he— well, he was diagnosed as autistic, but then they figured out that he also had an eye issue that could be helped with vision therapy. So they took him to vision therapy and, uh, he was able to progress so much that he graduated from the autistic program that he was in with the school system. They said he doesn't qualify as being autistic anymore.

David Kaufer [00:30:40–00:30:42]

Interesting. Yeah, that's great.

Denise Allen [00:30:42–00:30:48]

I, I think that that just kind of points to the fact that there's so much more going on.

David Kaufer [00:30:48–00:30:51]

There's so much more going on. Absolutely.

Denise Allen [00:30:51–00:31:16]

And, and we, we, we can look at the symptoms and say, well, it's got to be this, but that may not be the case, you know. And so there's a lot of misdiagnosis. of ADHD and dyslexia and all of those learning issues that we really should check the vision to see if that's what's causing the problem or if it can help the problem.

David Kaufer [00:31:16–00:31:18]

Right, for sure.

Denise Allen [00:31:18–00:31:19]

It's all working together.

David Kaufer [00:31:19–00:31:40]

And it seems that there is not a big emphasis on vision screening, really. I mean, it kind of— it's Almost like kind of a last resort. And do you think, again, is that lack of awareness, Denise, or is it just resources? I mean, what would you attribute? Would you—

Denise Allen [00:31:40–00:32:06]

There's so much that goes into that. Such a loaded question. Okay. I had to remove myself from the school system because it was such a hot topic for me personally. Because I wanted to go in there and I wanted to tell every parent, you know what, if your child is struggling in school, you need to take them to a developmental optometrist, not a regular optometrist.

David Kaufer [00:32:06–00:32:06]

Right.

Denise Allen [00:32:06–00:32:25]

Because those guys tell you everything's fine your whole life. That's what happened for me. That's what happened to my daughter. I specifically asked my optometrist if she had a problem with depth perception, and he told me she was fine. So you can't even get this kind of a diagnosis at a regular doctor's office.

David Kaufer [00:32:25–00:32:25]

Interesting.

Denise Allen [00:32:25–00:32:52]

And they're not going to screen for it in a school system, right? They're only screening for acuity, you know, at the eye chart across the room. There are things they can do that are not that hard, but they're not equipped all the time to do it. And what I found when I said, you know, I really want to tell this parent that they should take their child to the correct eye doctor, the school system said you can't tell them.

David Kaufer [00:32:52–00:32:55]

Why? What? Why not?

Denise Allen [00:32:55–00:32:58]

Because the school doesn't want to have to pay for vision therapy.

David Kaufer [00:32:58–00:33:41]

Yeah, it comes, it comes down to finances. So they don't, they don't, they aren't even questioning— I'm drawing a parallel because in terms of like in the non-speaking world, the There are issues with schools not allowing the use of letterboards or communication partners in there. And again, part of it's financial. Part of it in this case is also claims that, you know, it's not effective. I mean, there's a lot of other excuses they use in this, but I was curious.

So with this, though, you think it's purely financial? Do they only— they don't care if it's effective or not. They just care about the cost.

Denise Allen [00:33:42–00:33:50]

Well, they're not trained to do it, and they're— they don't have the funds to pay for, you know, a developmental optometrist to do it.

David Kaufer [00:33:50–00:34:20]

But they could still recommend it, couldn't they? I mean, couldn't they at least educate? I mean, I don't see the harm in at least— I understand them saying, hey, we can't provide this necessarily if this is an additional cost, additional burden. I mean, I know financially, right? I mean, our We ask a lot of our schools and stuff, and that's another topic, but I can at least understand that from a financial standpoint if that's just an additional cost burden. But to at least say, hey, here's something you should look at.

Denise Allen [00:34:20–00:34:41]

I think it's a big problem. Yeah, that's, that's why I'm saying I had to just not be in education, you know, and that kind of pointed me towards what I ended up doing. Right. At the end of the day, that's why I'm doing podcasting, because we need awareness.

David Kaufer [00:34:41–00:34:47]

So that really— what that really did, just kind of— because you were teaching, right? You were substituting?

Denise Allen [00:34:47–00:34:49]

I was substituting. Yeah.

David Kaufer [00:34:49–00:34:53]

I mean, you had 6 kids. I'm sure you've been busy. It's hard to juggle it all. But yeah.

Denise Allen [00:34:53–00:35:14]

Yeah. No, in the meantime, I had to find something that was going to pay the bills. Right. So instead of going the route of alternative route to licensure and getting a teaching certificate and being in the school system. I got my insurance license. I sold insurance for 10 years.

Plot/Intro [00:35:14–00:35:15]

Wow.

Denise Allen [00:35:15–00:35:29]

Finished raising my children and all of that. And in the meantime, you know, that gave me flexibility to focus on my own healing, to do my vision therapy, to get my child in and, you know, get all that handled. Get them all married off.

David Kaufer [00:35:29–00:35:31]

That's good.

Denise Allen [00:35:31–00:35:35]

Um, then I was able to spend more time on my podcast.

Plot/Intro [00:35:35–00:35:37]

And how—

David Kaufer [00:35:37–00:35:41]

And remind me, how long have you been doing your podcast?

Denise Allen [00:35:41–00:35:44]

I started it in 2020, about the time—

David Kaufer [00:35:44–00:35:47]

Holy cow, good for you! Wow, that's amazing.

Denise Allen [00:35:47–00:36:02]

Yeah, well, and I started it before I had everybody out of the house, and there were some Downtimes, you know, I think every podcaster gets to the point where they're questioning whether they're able to continue, right?

David Kaufer [00:36:03–00:36:03]

Sure.

Denise Allen [00:36:03–00:36:12]

And my point in time happened when I had 3 daughters get married in the space of 4 months in the summer of 2021. I blame that on COVID.

David Kaufer [00:36:12–00:36:16]

COVID marriages.

Denise Allen [00:36:16–00:36:22]

Well, they were all dating from the dating apps and stuff, you know. And they also—

David Kaufer [00:36:22–00:36:28]

Don't they have planned that a little bit, spread it out a little bit for your sake? I mean, geez, that's a lot.

Denise Allen [00:36:28–00:36:56]

You would think. Yeah. So that happened. And I had my third daughter get married by September of

that year. And then the— and I had one podcast come out that year, you know, in that summer. And then I had some technical issues and, and I just let life get to me, you know, the whole, now I'm an empty nester. What's my life look like? And then in April of 2022, I actually lost my husband in a motorcycle accident.

David Kaufer [00:36:56–00:36:57]

Oh, no, I'm so sorry.

Denise Allen [00:36:57–00:37:09]

And so that kind of shut me down as a podcaster too. So people look at my dates on my podcast, there's this huge gap in there.

David Kaufer [00:37:09–00:37:10]

Well, understandably.

Denise Allen [00:37:10–00:37:28]

And then I finally had to just say, you know what, this is why I'm still here, part of why I'm still here. Is because I still have work to do. I mean, there's a reason why he had to go and I had to stay, you know. So I got back into it.

David Kaufer [00:37:28–00:37:43]

What are some of the lessons you've learned? I'm curious, from podcasting, first of all, and some of the guests that you've had on there. What have— what are— or what are a couple of maybe surprising things that have come from it? That journey?

Denise Allen [00:37:45–00:37:54]

Surprising is a good question. I started to see how the whole body fits into the picture.

David Kaufer [00:37:54–00:37:55]

Yes.

Denise Allen [00:37:55–00:38:27]

Because if we look at it just from the eye standpoint, we limit ourselves, you know. And if we look at it from a whole body perspective, then we can say, okay, what is the real cause of this problem? Is there an alignment problem in the body that we can look at that will help us along the way, or that is signaling that there's something going on with the eyes even? I— this is a great example of my— from my own life. I was born pigeon-toed.

David Kaufer [00:38:27–00:38:27]

Yeah.

Denise Allen [00:38:27–00:38:39]

So we already have a misalignment, right? The way they handled that back in my day, since I'm so old, is that they, they would put casts on your— on the baby's feet.

David Kaufer [00:38:39–00:38:41]

Mhm. I remember hearing about that.

Denise Allen [00:38:41–00:39:08]

Mhm. For like 6 weeks. And then, uh, after the cast comes off, then they put a bar with 2— with the orthopedic shoes, right? And the shoes are pointing out, and, and you have to wear that, that— and most of the time, you know, protect— I remember it being during naps and stuff, you know, like it was, it was a thing that Well, I had to deal with my oldest daughter too. Like, my daughter had the same problem.

David Kaufer [00:39:08–00:39:10]

Oh, wow.

Denise Allen [00:39:10–00:39:46]

I know. So there's that misalignment that I think you should pay attention to, you know, because the body's telling us something, right? It's telling us that we need to pay attention to what's going on as far as I take it personally, I take it back to feelings too. But with my guests, I have talked with them about the fact that there's misalignments in the body that we could address as well. So it's kind of a long

answer. I don't know if that gives you exactly—

David Kaufer [00:39:46–00:41:04]

It does. Yeah. No, I know. I'm just kind of thinking as you're talking about that, you know, the whole body approach. I think it makes so much sense.

And I think it's something that we've continue to learn about, you know, as people in the medical profession. It's kind of getting back to what you said earlier. It's kind of funny. Can we start? The topic was talking about kind of the vision therapy.

And then, of course, you think about just the eyes, right? This is just a thing that has to be. And we do that with our body, right? It's just— And in healthcare in general, we isolate things at just one thing.

Oh, you have a problem with headaches or whatever.

So it's just, you know, it's a headache problem. And, you know, where could we find out later could be something related to a spine issue, you know, misalignment. That's everything. To your point, everything is so much more connected than I think we're taught or realize or assume or whatever. I'm, you know, I'm— again, I'm not sure.

It's kind of like one of those systemic things. And I'm sure— I think some of it's probably just kind of the, you know, traditional healthcare approach that the specialists— in our age of specialists, you kind of choose a specialty. And so, right, I don't know.

Denise Allen [00:41:04–00:41:06]

And they don't talk to each other.

David Kaufer [00:41:06–00:41:07]

And they don't talk, right.

Denise Allen [00:41:07–00:41:33]

And in our world today with vision, even the ophthalmologists and the vision therapy doctors don't typically talk to each other either, which I think is malpractice, honestly, because there's so much good that can be done when they collaborate. You know, if I hadn't had vision therapy, surgery, and more vision therapy, I would not have gained 3D vision.

David Kaufer [00:41:33–00:41:34]

Right.

Denise Allen [00:41:34–00:41:57]

But there's so few doctors that work together, you know, and if you go in and you only have the surgery, the percentage of success isn't even being measured. If you want, if you want the result to be 3D vision, they're only measuring whether the eyes are aligned. So they consider it a cosmetic surgery.

David Kaufer [00:41:57–00:41:59]

So limited cosmetic surgery, right? That makes sense.

Denise Allen [00:42:00–00:42:00]

Wow.

David Kaufer [00:42:01–00:42:18]

Why don't they talk more? Is it ego? Is it just— is— I mean, is there— is it, is it a turf battle? I mean, honestly, I don't know. I mean, again, we see this in other areas, right? And I'm drawing analogies to, you know, the world I'm most familiar with in the autism world. But in this world, what do you see, Denise?

Denise Allen [00:42:18–00:42:39]

Um, I, I think it's an ego problem personally. You know, I, I think they're so caught up in where's the scientific proof, you know. Well, who's going to do these studies that prove that you're gaining 3D vision? You know, it's—

David Kaufer [00:42:39–00:42:41]

Aha. So that's—

Denise Allen [00:42:41–00:43:00]

Is that kind of their Yeah, they'll say there's no studies that prove it works. Well, there's all kinds of

people who can say, look, this worked for me, you know, but everyone's problem is so specific. Doing double-blind studies doesn't even work. I mean, there's— how would you even do a study like that?

David Kaufer [00:43:00–00:43:02]

You can't, right?

Denise Allen [00:43:02–00:43:08]

You can't find people whose situation is exactly the same. No one's is. So you can't do a study on it.

David Kaufer [00:43:10–00:43:40]

That's so interesting. Um, and again, that— I was just gonna say, Denise, that's the connection. That's exact— because that's exactly the battle that parents and, you know, nonspeakers are fighting as well in our world. And when talking about the world of letterboards and, you know, we call it spelling to communicate and, and all this, the establishment, in this case the Speech and Hearing Association, Therapists Association, they use the exact same argument. There's no, you know, this isn't scientific.

Denise Allen [00:43:40–00:43:41]

Mm-hmm.

David Kaufer [00:43:41–00:44:05]

There's no scientific proof. And the response is, you know, the proof is in what you're seeing. I mean, people are using it successfully, and it sounds very similar to what you're saying with vision therapy. So, so, so it sounds like kind of traditional eye doctors see vision therapy as Do they see it as like woo-woo or kind of—

Denise Allen [00:44:05–00:44:31]

Almost. Yeah, I think they do. There's so many times that they just say, oh, it doesn't work, or you're not a candidate, or they don't even mention it to someone who's been in for eye surgery. They'll say, well, this surgery might not work. You might need another one. I mean, I'm on these support groups for strabismus on Facebook, and it's, it's a constant thing.

David Kaufer [00:44:31–00:44:33]

Wow.

Denise Allen [00:44:33–00:44:35]

They just, they don't tell him about it.

David Kaufer [00:44:35–00:45:08]

Yeah, that's like I said, that's, um, that blows me away in a way, the parallels that we see. And you don't, you don't know what you don't know. So going back, because, you know, one of the obvious questions would be, Denise, is for parents, what should they be looking for? I mean, if you're, if your eye doctor is not going to give you a heads up like, hey, here's, you know, here's Here's something to look for. If they're not going to steer you to vision therapy or, you know, some of these other, you know, treatments, what, what should parents be looking for?

Denise Allen [00:45:08–00:45:20]

Well, I kind of alluded to it a little bit with what I mentioned about my daughter's story, you know, asking those specific questions if your child is verbal and can tell you, right?

David Kaufer [00:45:20–00:45:21]

Right.

Denise Allen [00:45:21–00:45:37]

Obviously, it's way harder if your child's not verbal and they can't tell you, but Um, I have a little checklist that I made for, uh, parents. Again, it's gonna, it's gonna be a little harder depending on your child, but if they're—

David Kaufer [00:45:37–00:45:41]

Yeah, but no, is it on your website or do you, do you have—

Denise Allen [00:45:41–00:45:44]

Um, yeah, and I can, I can go over the shortlist. It's a very short—

David Kaufer [00:45:44–00:45:45]

Yeah, I want to hear it.

Denise Allen [00:45:45–00:46:15]

Yeah, so, um, they lose place or skip lines or reread something, and when they're reading Um, they avoid reading or they get frustrated really quickly. Um, they would understand better when they're listening than when they're reading. They have a short attention span for schoolwork, but if they have a preferred activity, their attention span is long. Um, they get tired or emotional or overwhelmed during learning tasks.

David Kaufer [00:46:15–00:46:17]

Mm-hmm. Right.

Denise Allen [00:46:17–00:46:41]

They're clumsy like I was. They bump into things or they struggle with coordination. Anytime you've been told everything is fine and you still have concerns, that's the time to go and have it checked out, you know. But the thing is, is that a regular optometrist or an ophthalmologist is not going to look for the same kinds of things as a developmental optometrist will look for.

David Kaufer [00:46:41–00:46:41]

Right.

Denise Allen [00:46:43–00:46:45]

Right. So you really have to go to the right doctor.

David Kaufer [00:46:45–00:47:07]

No, that makes a lot of sense. Now, this is kind of— this may be kind of a dumb question, but can— if, if, if your child has, you know, this type of vision, and I mean, would they still be able to appear normal, you know, that they have normal vision other ways, like playing video games or things like that? Is that affected?

Denise Allen [00:47:09–00:47:13]

Yeah, I mean, a video game isn't going to have a depth perception requirement.

David Kaufer [00:47:13–00:47:16]

So that's easy. That's easier because it's not— okay.

Denise Allen [00:47:16–00:47:25]

Yeah, I mean, it's, it's, it's in 2D most of the time, you know, unless you're playing with the VR headset, you know.

David Kaufer [00:47:25–00:47:26]

VR headsets, right.

Denise Allen [00:47:26–00:47:31]

Which actually a lot of vision therapy doctors are starting to use the VR.

David Kaufer [00:47:31–00:47:34]

I was wondering about that, if that would be helpful with that.

Denise Allen [00:47:36–00:47:49]

Yeah, I actually even have one, and I, I've been so anti-video games that I can't make myself do it, but I bought one. I was like, I'm gonna do this, and I haven't. But it is something that a lot of patients find very helpful.

David Kaufer [00:47:49–00:48:16]

Yeah, no, you've just— you've got me thinking in terms of— and I'm just thinking, are there, are there any other questions that you can ask your child again that maybe aren't reading related, or things— I think about, again, our audience who may be have nonspeakers, but it can communicate through typing. Um, are there any other things that they can ask or ways to try to determine if this is a problem or a potential problem or issue?

Denise Allen [00:48:17–00:48:48]

The things I'm seeing on these checklists are going to have to do with, um, being able to keep your place, you know, so the hand-eye coordination piece could apply to nonverbal, you know, if they're If

they have a hard time even picking something up or catching or whatever, right? That's a hand-eye coordination issue. I remember Jillian saying that she used to stick out her pinky when she was grabbing the cup, you know, because it helped her orient where her hand was going to go to grab it.

David Kaufer [00:48:48–00:48:49]

Where her hand was, yeah.

Denise Allen [00:48:50–00:48:56]

Interesting. So you would see compensations in that hand-eye coordination piece.

David Kaufer [00:48:59–00:49:10]

Okay, great. Well, let's, um, let's move on to our weekly segments because I know that you are prepared. Thank you, Denise, for coming prepared.

Denise Allen [00:49:10–00:49:10]

Sure.

David Kaufer [00:49:10–00:49:52]

Yeah, and so we're going to start off, and for each of these, just give you fair warning, we do have little jingles that will— that Dave, producer Dave, will play. And then we'll get— we'll start off with the tip of the week. It won't necessarily be Autism Tip of the Week, but it'll be Denise's Tip of the Week. Hey parents, gather around, it's time to speak. We're here with your tip of the week, strategies and wisdom just for you.

Autism parents, we know what you go through. All right, it's time for our tip of the week, and as I mentioned earlier, I know that Denise, you brought one for us. What do you have?

Denise Allen [00:49:53–00:50:10]

Well, we've kind of talked about it a little bit already, but I'm just pointing to the fact that a child may be able to see clearly and still have difficulty using their 2 eyes together. And a regular vision screening, even at a doctor's office, may not tell the whole story.

David Kaufer [00:50:10–00:50:22]

Okay, that's a great— that's a great tip. And I'm just going to ask kind of a related tip. If every parent could know one thing about binocular vision, what would it be?

Denise Allen [00:50:26–00:50:28]

Well, not everyone has it.

David Kaufer [00:50:28–00:50:29]

Okay.

Denise Allen [00:50:29–00:50:29]

You know?

David Kaufer [00:50:29–00:50:30]

Yeah.

Denise Allen [00:50:30–00:50:39]

I think we just take that for granted, not just as a parent, but as a person alive on the earth today. We just think everyone sees like we do.

David Kaufer [00:50:39–00:50:42]

Yeah, right. You just assume that.

Denise Allen [00:50:42–00:50:56]

I've talked to so many people, they don't even know what binocular vision means. And I say, well, I didn't see in 3D before. And they say, well, do I see in 3D? I said, well, Probably. I don't know.

David Kaufer [00:50:56–00:50:57]

I can't tell you.

Denise Allen [00:50:58–00:50:59]

I can't see like you do.

David Kaufer [00:50:59–00:51:06]

Yeah, that's funny. All right, next up we have Fact or Fiction.

Denise Allen [00:51:06–00:51:17]

Get QuickTime ready, and here we go. This is Autism Fact or Fiction. We're breaking down what's real, what's rumor, and what might surprise you. Let's go.

David Kaufer [00:51:17–00:51:32]

All right, so this is again where I like to introduce potential stereotypes or myths or, you know, things that may misperceive— misperceptions that may be out there. But, um, Denise, what do you have for fact or fiction?

Denise Allen [00:51:32–00:51:48]

Well, again, we've talked about a lot of this, so I'm kind of feeling like maybe this is just a review. Uh, the fact or fiction that I chose was, uh, 20/20 means that your vision is fine.

David Kaufer [00:51:50–00:52:07]

Right. I think that's a big one, though. I think that bears repeating because I think that's such a— that I think that's just presumed to be fact. I think that's just so ingrained in us that that's the goal. It's just all about 20/20 vision that you don't even think about that.

Denise Allen [00:52:07–00:52:18]

Right. And because my glasses corrected my vision so that it was 20/20, I must be fine, right? Yeah.

David Kaufer [00:52:19–00:52:29]

Yeah. And I guess kind of related to that would be, you know, I jotted down, you know, like my child passed their vision screening, you know, at school. So everything's fine.

Denise Allen [00:52:29–00:52:30]

Yeah.

David Kaufer [00:52:30–00:52:32]

Not true.

Denise Allen [00:52:32–00:52:35]

And being told that you're fine for years and years and years.

David Kaufer [00:52:35–00:52:37]

Oh, yeah.

Denise Allen [00:52:37–00:52:39]

And knowing that you're not really.

David Kaufer [00:52:41–00:52:51]

I mean, there's, there's a little bit of trauma that goes with that, right? I mean, kind of, isn't it a little bit of being like gaslit for so many years? Like, you're like, I know this isn't true, but I can't prove it.

Denise Allen [00:52:51–00:52:55]

Yes. And I've heard it from many of my guests as well.

David Kaufer [00:52:55–00:53:00]

That they knew, that they knew something was wrong, but—

Denise Allen [00:53:00–00:53:23]

Yeah. And, and one in particular had the cerebismus surgery. And went back to her surgeon and said, you know, I'm really struggling. She had a complete breakdown of her visual system after her surgery. You know, you move the eyes around, she's used to compensating a certain way, and her eyes had no idea what to do at that point. And this was—

David Kaufer [00:53:23–00:53:23]

Wow.

Denise Allen [00:53:23–00:53:33]

A while after the surgery that she had this complete breakdown, and they treated her like she was crazy, like she should just— but they literally said, It's all in your head.

Plot/Intro [00:53:36–00:53:36]

Unbelievable.

Denise Allen [00:53:37–00:53:43]

It was all in her head because it was her brain not knowing how to tell her eyes how to work together.

David Kaufer [00:53:43–00:54:17]

Right, right, right. Now I go back to the, the point you made right off the bat, Denise, in terms of how our eyes are muscles. And if you're going to have surgery on a muscle, it's just like anything else. Like, I can't imagine someone who goes through, say, you know, again, sports-related, that people relate, you know, can go through ACL surgery and just like, you just have surgery and then say, okay, you're good to go without PT, right? Without any type of follow-up. I mean, it sounds like that's kind of the expectation with, with eye, the eye surgery.

Denise Allen [00:54:17–00:54:29]

It is. Yeah. And that's actually one of my favorite analogies. You would never have surgery on any other part of your body and not have physical therapy before surgery or after surgery or both.

David Kaufer [00:54:30–00:54:30]

Right.

Denise Allen [00:54:30–00:54:35]

Right. And yet that's what we are doing all the time with vision, with our eyes, with our eye surgery.

David Kaufer [00:54:35–00:54:57]

That's crazy. Sorry, that really is crazy. I mean, that's just, um, I guess it just seems— yeah, it seems like there's— we need to progress the view there, and as we have in so many other areas of medicine and healthcare. I'll put it that way. Saying it's just crazy isn't very productive.

Denise Allen [00:54:58–00:55:07]

Well, it's, it's not, but the awareness is what we're trying to do so that people will know that there needs to be a change. You know, we, we have to demand it of the doctors.

David Kaufer [00:55:07–00:55:50]

Right. Yeah. And that's, yeah, and that's kind of like a whole nother conversation, right, Denise, is changing our mindset, um, as patients in general, as advocates for our kids, you know, dealing with doctors, um, Trusting them as experts, which they are. They're trained. They're, you know, they have a lot of education and training, which deserves respect.

But that doesn't mean they're all-knowing, all-knowing, and they, and they don't know our bodies. I mean, they can't. At best, a lot of what they're doing is still kind of making educated guesses about the approach, which, you know, that's just the way it works.

Denise Allen [00:55:51–00:56:00]

But— And it's true in the vision therapy room, too. They have to figure out where the missing piece is, you know?

Plot/Intro [00:56:00–00:56:00]

Mm-hmm.

David Kaufer [00:56:00–00:56:01]

Right.

Denise Allen [00:56:01–00:56:26]

And you can talk about vision therapy with relation to traumatic brain injury and everything, too. And, you know, I've had many of my guests that work with that population say, if you've treated one person with a traumatic brain injury, you've treated one person with a traumatic brain injury, you know? So there's just a lot of things going on with that, you know? And I think some of the autistic population falls into that traumatic brain injury category too, right?

David Kaufer [00:56:27–00:56:27]

Yeah.

Denise Allen [00:56:27–00:56:32]

And so it's just they're practicing medicine, you know, they're, they're trying to figure it out.

David Kaufer [00:56:32–00:57:31]

Yeah, for sure. No, I mean, it's, it's, it's fascinating. All right, well, next up on our segment is going to

be our confession of the week. Parent confession. Morning routines can go All right, now's the time where we get to As I always say, share our humanity so that we don't just have the answers. We've made mistakes too. And Denise, I mean, with 6 kids, I'm sure there's a couple of things that may have come to mind. But what— yeah, what would you like to share with everyone?

Denise Allen [00:57:31–00:58:02]

Well, you know, I already talked about my daughter Abby and how I didn't know that she needed vision therapy till I knew, right? Um, I don't talk about this a lot, but I actually had another child who had some struggles and still, still does, actually. Um, there's another vision issue called convergence insufficiency. And, um, my son had been struggling in school.

David Kaufer [00:58:02–00:58:03]

Mm-hmm.

Denise Allen [00:58:03–00:58:15]

We, we thought that he was just lazy because he could read, he could do the work. If he did the work, he got 100%. He got an A. If—

David Kaufer [00:58:15–00:58:16]

Uh-huh, right.

Denise Allen [00:58:16–00:58:18]

Most of the time he didn't do the work. Most of the time.

David Kaufer [00:58:18–00:58:19]

Okay.

Denise Allen [00:58:19–00:58:33]

And it was not until he was 15 that we had him diagnosed. And my doctor said it was probably the worst case of convergence insufficiency he'd ever seen.

David Kaufer [00:58:33–00:58:37]

Oh, wow. And what, what is it? Can you give a—

Denise Allen [00:58:37–00:58:56]

Convergence insufficiency means that when you try to focus close, Your eyes just give up. You can only sustain the convergence, the eyes working together close up for a limited amount of time before it's just too tiring. And it's—

David Kaufer [00:58:56–00:58:58]

It's like an overload, probably, right? Yeah.

Denise Allen [00:58:58–00:59:19]

And you just don't want to do it, right? Which is so why he wouldn't focus on his work, you know, just didn't want to do it because it was so hard. to do it. And I did. I took him to vision therapy. He did several months of vision therapy, and he was just not really progressing with it, you know.

David Kaufer [00:59:19–00:59:20]

Okay. Yeah.

Denise Allen [00:59:20–00:59:27]

And so, you know, by that age, it's really hard to force them to do it.

David Kaufer [00:59:27–00:59:28]

Sure.

Denise Allen [00:59:29–00:59:29]

And I—

David Kaufer [00:59:29–00:59:30]

To do anything.

Denise Allen [00:59:30–00:59:51]

I gave up. That's, that's my big confession, actually. I don't— I hardly ever talk about it. Yeah, it's still — I mean, he doesn't— if he wants to do something, he figures out how to do it. He's very smart. He can do whatever he wants to, but schoolwork is not it.

David Kaufer [00:59:51–01:00:01]

He's— yeah, it sounds like he's found other, other ways of coping and other ways to get things done. But he also knows that that option's there, right?

Denise Allen [01:00:01–01:00:10]

He does, yeah. And I've told him that many times, you know, you can go back and do the vision therapy and get the results if you want to.

David Kaufer [01:00:10–01:00:51]

Yeah, well, that I could see where that, that would be really difficult. But there's only so much that you can do too, right? I mean, that's, that's the part that I think we all struggle with as parents is, you know, we want to be able to control as much as we can. We want to be able to boost our kids and help them. I was just having that conversation with my brother, um, the other day.

It's, it's a, it's a really tough balancing act between, like I said, pushing too hard or, you know, and pulling back too far too. And unfortunately, there's no, uh, there's no perfect answer, right? You just do it. You just do what you think is best.

Plot/Intro [01:00:51–01:00:51]

Yeah.

David Kaufer [01:00:53–01:01:08]

But yeah, thank you. Thank you for sharing that. That's— I mean, I really appreciate that. Well, we're going to turn it around 180, and again, because we like to end this on a lighter note, so we have our semi-news segment, which is Light on the Spectrum.

Denise Allen [01:01:08–01:01:29]

Light on the Spectrum. A moment, person, or story bringing something good. It feels Hopeful, grounded, not performative. Use it for a person, a moment, a breakthrough, a story.

David Kaufer [01:01:29–01:01:39]

All right, so like I said, what do you have for us, Denise, to give, give our listeners, parents out there, a little bit of hope?

Denise Allen [01:01:39–01:02:04]

Well, I think that my story is my biggest gift, right? Because neuroplasticity is a real thing. You know, you don't have to say, oh, this is just the way it is. And I mean, I've heard it so many times. People will say, I have permanent double vision. I will never see in 3D.

David Kaufer [01:02:04–01:02:05]

Mm.

Denise Allen [01:02:05–01:02:56]

Happens all the time, you know. And when I went into my surgeon for my first visit with him and I told him that my goal was to see in 3D, he didn't take that super well, really. You know, he, he actually told me that he wanted me to come back in and do some more testing because they were getting some new machine in there that was going to be able to gauge whether I'd be successful, that kind of thing. And I already knew I was going to be successful.

But he didn't, you know, and so I waited and I went back in and I did their other testing and stuff and found out that he actually, and I'm, it probably shouldn't share, but I knew he, that he thought that I was being unrealistic.

David Kaufer [01:02:56–01:03:05]

Right. Sure. He's being overly, overly cautious or overly skeptical or choose your, choose your word. Right.

Denise Allen [01:03:05–01:03:13]

I mean, right. Right. But I mean, neuroplasticity says our brains can change, right? The way we see—

David Kaufer [01:03:13–01:03:14]

100%.

Denise Allen [01:03:14–01:03:24]

The way we see can change. The way we do things can change. Our bodies can improve in many different ways, right? We see that all the time at every age.

David Kaufer [01:03:24–01:03:25]

Right.

Denise Allen [01:03:25–01:03:41]

Like, it doesn't matter if you're 50 or 70 or 20, right? It can change. And so My gift is that I've proven it can change. You know, I gained 3D vision at age 54.

David Kaufer [01:03:41–01:04:15]

That's amazing. No, that's a great message that should give a lot of people hope. And like you said, especially the message about neuroplasticity and just how it can't be emphasized enough because there's so many contexts that we hear that, that like you said, I have permanent XYZ. Um, for those of us like in an autism world, you, you know, we've dealt with a lot of negativity. We've— and now we're seeing a lot of positives. So, um, before we wrap up, Denise, how can people find you?

Denise Allen [01:04:15–01:04:27]

Well, I have my podcast on every podcast player, and recently I put it on YouTube. It's got a static image. I don't do video because I'm not that great.

David Kaufer [01:04:27–01:04:27]

Okay, okay.

Denise Allen [01:04:28–01:04:49]

But, uh, well, I just haven't Felt like I could edit a video, right? I'm good at editing audio. So, but it is on YouTube, it's on all the podcast players, Healing Our Sight. Um, I have a Facebook group also called Healing Our Sight and, um, website healingoursight.com.

David Kaufer [01:04:49–01:04:49]

Awesome.

Denise Allen [01:04:49–01:04:53]

Yeah, so they can reach me in any of those ways.

David Kaufer [01:04:53–01:05:46]

Great. Well, thank you, Denise, for joining us today. I really enjoyed the conversation. I know that our listeners and viewers will walk away looking at a And eyes, maybe even the brain a little differently. And thanks to all of you for spending part of your day with us.

If you enjoyed today's episode, please subscribe, leave us a review, share it with someone you think would benefit from the conversation. This is one of the best ways to help us reach more families, continue bringing these important stories to a wider audience. I'd also like to give a special thank you to our producer, my guy Dave Yaz, and the entire team at Pod Save 617. Dave's been with me since the beginning of our podcast. We're coming up on 2 years.

His expertise, creativity, and friendship have helped make the lighter side of the spectrum what it is today. So again, thank you, Denise, and we'll catch everyone else next week.